

I: Okay.

I: Let's continue with the next section.

I: The first question is about familiarity with AI. How familiar are you with AI technologies in general?

P: Not very much. I know ChatGPT, but that's about it. But that's about it, I'm not very tech-savvy.

I: Have you used ChatGPT?

P: Yes.

I: For what kinds of things?

P: For study work and for my work right now. Mostly for advice and for trip planning.

I: What type of advice?

P: If I observe a behavior in a child that I don't have much knowledge about, I consult ChatGPT and ask for ideas about what interventions might be useful.

I: So you are not using AI within your organization?

P: No, they are very old school.

I: Did you disclose to your clients that you sometimes use AI for consultation?

P: No, I don't think so

I: On a scale from 1 to 10, how familiar do you consider yourself with AI, where 1 is very low and 10 is very high?

P: With AI? Probably a 3 or a 4.

I: Okay, we'll move on to the next question.

I: This question is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think it can be quite useful because it's more objective than humans are. But it should be used in parallel with a human professional, not on its own. But I think generally, I think there is a lot of potential there.

I: Do you currently do diagnostics yourself?

P: No. I'm a psychologist, not a psychotherapist, so I'm not allowed to do diagnostics and testing.

I: Okay. You still think it might be useful, right? And you're saying it's more objective?

P: Yea, because sometimes what people do is put labels very easily. I can see that I have a colleague, right, and as soon as she sees a child she's like, "Oh, that child is surely going to develop a personality disorder." And I don't see a lot of grounds for that or a lot of evidence yet. And I think if that was an AI, they wouldn't draw these conclusions so easily. They would have more objective parameters and not just a gut feeling.

P: You know what I mean?

I: So you're feeling that sometimes psychologists just think, "Oh, I already know what's up," and they'll come up with some story in boxes and labels.

P: Yeah, definitely. And I think AI would be more cautious with that.

P: And with more scientific parameters, not just gut feeling.

I: Okay, so intakes, have you considered ...?

I: Do you know if you're doing any preliminary screening before the intakes?

P: No, I don't do that.

P: But we have a psychologist who's my subordinate.

P: She does that.

P: But I don't.

P: I just get the children.

I: And how do you do the screening?

I: Do you know how it goes?

P: Um...

P: Yes, so what they do is that, for example, a school or a youth worker can hand in an application for a child. Then the psychologist at the institution goes through the application, and they invite the family with the child and the youth worker or whoever submitted the application for an interview.

P: Then they ask background questions and goals for the time being with us. Goals of the child, goals of the person submitting the application, and the parents and stuff.

P: This is what they do for screening.

P: But yeah.

I: But that's done in person, right?

I: Pardon?

I: That's done in person, right?

P: Yes, that's done in person.

P: It's not with AI at all.

I: Yeah, but it's also not like a questionnaire that they fill out.

I: It's actually more like a conversation.

P: I think there is sort of a questionnaire, but it's more like what types of problems the child has.

I: It's semi-structured.

P: What are their strengths and resources?

P: I'm not sure when they deliver that to the family or the youth worker or the school.

P: But we get a folder with this information when the children arrive here.

I: So have you considered AI for doing those screenings?

P: I haven't, because I don't do the screening.

P: And I'm not entirely sure, because I'm not so much in touch with the people who do that part of the job.

P: But I think they want to make some changes generally in the organization, so that things become more digitalized.

P: But I'm not sure if that also applies to the screening.

I: Yeah.

P: I'm not sure. Sorry.

I: Okay, that's fine.

I: And what about summarizing patient information?

I: For example, in an intake you talk a lot, right?

I: But afterwards you probably have to summarize quite a lot.

I: Do you think AI would be able to do that?

P: With AI? I think that would save a lot of time.

P: Because documenting is a lot of work and takes a lot of time and effort.

P: I don't do it with AI. I just summarize my notes.

P: But I think it would be much more convenient if we did that.

P: I just wouldn't know how that could be implemented because of data protection.

P: My organization is very strict about data protection.

P: So you need to be really careful how you store information about the children.

P: Yeah.

I: Yeah.

I: So you're a little bit reluctant because of privacy issues.

P: Yeah.

P: They are.

I: Okay, let's move on to the next question.

I: It's about monitoring and progress tracking.

I: How do you view the role of AI in tracking patient progress over time?

P: I think it could also be very useful.

P: Because you have more objective tracking and information collection that can be stored more efficiently and compared.

P: But what's complicated is the parameters you choose.

P: What is progress?

P: What is success?

P: How do you measure that?

P: I think that can be quite complicated.

P: Right now, I don't really see a way how that could be done.

P: But I'm also not so much into research anymore, so I don't know how far things are in that regard.

I: So do you do any monitoring or tracking?

P: Yes and no.

P: Like we have statistics.

P: So what we did with the children in terms of interventions.

P: And that's also how many children, this is a program that lasts four weeks, so the children can stay here, but they can also leave early. Or if there are extreme conflicts and escalations, some children have to go back home if they endanger themselves or other children.

P: So that is also in the statistics: how many people completed the program and how many people left early.

P: That is what we do in terms of progress tracking.

I: It's just general population tracking. It's not on an individual basis?

P: No. no

I: Let me think—so you're not tracking progress per person?

P: Yea, more like what we did with all the children. Like how many interventions of type A we did, how many interventions of type B.

P: We do put that in a chart for every individual child, but it's just to get it summarized.

P: And that's going to be added up and included in a yearly report.

P: Basically it's more about justifying to donors why this is necessary and effective.

P: But there's no long-term tracking.

I: The patients are done either early if something happened, or if they're better or cured?

P: I wouldn't say cured. It's more like getting a break from whatever is burdening them at home.

P: It's sort of a restorative environment for four weeks.

P: I don't know if you have something like that in the Netherlands. In the UK they don't quite do that, or maybe under a different name.

P: It's sort of like a holiday but with a clear objective, psychological support, and pedagogical support.

P: I wouldn't call it wellness or anything like that.

P: But for example, if you have health conditions like respiratory diseases, you can also go to an institution for four weeks to improve your health.

P: You're not being cured, you know what I mean.

I: Yeah, okay, that makes sense.

I: Then the next question is about administrative and practical support.

I: How do you feel about AI generating drafts or reports, or assisting with scheduling and reminders?

P: Oh yes, please. Please, please, please.

P: This is such a big issue in our organization, communication and exchange of information.

P: I think that would be the greatest aid this place could get.

P: Honestly, there are so many miscommunications and arguments resulting from faulty human information sharing or no information sharing.

P: I think AI would be such a blessing.

I: You said, this would be the greatest, and that it cut out a bit, what did you say?

P: That would be the greatest aid for our organization in this setting.

P: So much conflict happens between colleagues, and so much time and resources are lost because of human faults in communication and information distribution.

P: I think that would be such a benefit for all of us.

P: I think that would be awesome.

P: Yeah.

I: Would you trust AI-generated reports, or would you need to review everything carefully?

P: Yes, well, not 100%.

P: I would trust AI if I could review it afterwards.

P: I know it makes mistakes, but generally I think it does a good job.

I: Yeah.

I: How much time do you think this could save you?

P: So many hours. I can't really say how many exactly, but I work around 40 hours a week.

P: I think more than 10 hours, probably between 5 and 10 hours.

I: Okay, and another question: what concerns would you have about accuracy and liability?

P: That's a good question.

P: As I said, AI makes mistakes.

P: I'd be more concerned about data protection, especially with an AI program like ChatGPT.

P: My main concern is how the data is stored and used.

P: Otherwise, as long as I can check it, I feel in control and responsible.

P: I don't think it's the AI's responsibility.

I: Yeah.

I: So liability still lies with the psychologist using the AI?

P: Yes.

P: One hundred percent.

P: Yeah.

I: Okay.

I: Um, then for the next question, this is about psychoeducation and between session tools.

I: I'm not sure if this would apply to you, but we'll ask it anyway.

I: Um, so what are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think that's a great idea because it's, um, like psychoeducation can easily be delivered through an AI.

P: And I think it can actually do that very efficiently and in an engaging way as well.

P: And like in between session practice is generally very useful for repetition and for stabilizing behaviors or alternatives to behaviors and treatment.

P: So I think that would be really good because you can probably do that at low cost at home, very easily accessible.

P: So I think that would be great.

I: Yeah.

I: So are you doing any like between sessions?

I: Because I'm not really sure how your day-to-day looks, right?

I: But do you have any in-between session support or psychoeducation that you're already using with your clients?

P: Yes.

P: Sometimes I give the children homework for our next sessions.

P: And I think because, you know, this is Gen Alpha, right?

P: It's a very tech-savvy generation.

P: And I think it would...

P: They want to be constantly on their phone, which I don't think is good.

P: And we try to limit that by having phone times.

P: But I think in real life, if they weren't here, I think that would be really good if they could, in between sessions, access that through technology.

P: I think that would work for that generation specifically.

P: But also for people our age, older people, it's a bit tricky.

I: yeah so during your treatment they don't really have that much access to the phone

P: they have access to their phone, but in a limited way so some hours of the day they have the phone some hours they don't i could give them homework for when they're on their phone but i assume because here it's limited to some hours of the day that at that time they wouldn't want to do the

P: psychological homework but more be on the phone for fun for chatting for YouTube whatnot so I think that wouldn't work in this context but generally I think that would with apps and stuff I think there are very good apps for smoking cessation and stuff for smoking.

I: okay makes sense so what if the ai provides information that contradicts your treatment approach

P: then i think i would have the responsibility to look up the latest research and inform myself and if the ai is wrong i think there should be a way to sort of report that or

P: send feedback to the ai to correct that but still it's my responsibility i think to be aware of that and to educate myself further i think you cannot let the ai do your job completely

I: so would you worry about clients relying too much on ai between sessions

P: That's a good question.

P: I think that has to be I'm not quite sure how that would happen.

P: it depends on the client really if that would be to your benefit all the time or not but maybe that could be limited in a way for example in an app feature if you get what I mean or whatever it doesn't have to be an app but yeah I think you need to put some things in place to still I don't want to say limit the client or the patient but it depends on

P: the profile of the patient or the client i reckon i think for example if you have OCD or anxieties that could go the wrong way perhaps or an addiction pattern or something like that

I: And then limiting might just be time limiting or?

P: if you do an in-app session between your psychological sessions.

P: And once you've completed that, you cannot progress further.

P: It's just this is what you can do and no further.

P: Yeah.

P: Something like that.

P: But honestly, I have so little knowledge on AI and technology in general.

P: So maybe I'm just talking shit.

P: I don't know.

I: No, I mean...

I: I've had a wide range of people thinking they have knowledge on AI or they don't have knowledge on AI.

I: And I think it's most important that we figure out what you guys think.

I: Because then there's also...

I: If we find out that psychologists don't know anything about AI that's also something we found, so let me see so how would you monitor what the ai is telling your clients

P: Yeah, I think that has to be pre-approved.

P: Like there has to be a lot of rigorous testing before.

P: But that is difficult because an AI will probably generate individually, right?

P: That is a tough one.

P: I'm not sure.

P: I'm not sure.

P: Oh, that's a hard one.

P: Honestly, I'm not sure.

P: Hmm.

P: And I'm not sure how you know, like, an AI is developed, so I don't know how you could intervene in that process, right?

P: I don't know.

I: Okay, that's fine.

I: Okay, so then let's move on to the next question.

I: This is about the risks and limitations.

I: We've briefly mentioned a few, I guess.

I: But then if you want to extend on that, or if you want to explain more about them, so what risks or concerns do you have about using AI in clinical work?

P: Yeah, data protection, as I said, like not knowing where the data goes, who has control over the data.

P: I have a friend who's in law and she wants to work in that realm so she told me a bit about that that there is very little regulation actually so that is a concern I think and accurateness of information which you also mentioned in the previous question

P: still having control over what the AI delivers to a client or a patient.

P: That is tricky, I think.

P: And especially for the second one, I don't know.

P: I wouldn't know a way around that right now.

P: Yeah. also loss of jobs

I: so with the data privacy right so what are some of the information that you think would need to be very secure

P: like especially diagnostic information because in our society it's a fact that if this information leaks they can for example harm your career or basically any part of your life it's a bit like with medical information generally I think that is a concern and just vulnerability and stuff if the client trusts you with things these kinds of things

I: So in general, just any diagnostics or any vulnerability connected with identifying traits, right?

I: So the name or where they live or whatever.

P: Yeah.

I: Let's see this one.

I: So what concerns do you have about transparency and explainability of AI decisions?

I: So we kind of shortly already talked about how it's important that, actually did we do that

I: I'm not sure.

I: So what concerns do you have about transparency or explainability of AI decisions?

P: Yeah.

P: Explainability.

P: I think

P: what I've seen with ChatGPT that it can explain to you the decisions or the output that it gives.

P: So I think that makes me feel more secure with it.

P: Yeah.

P: But I think it would be good to have a second instance checking the responses constantly.

P: So to say like a feedback loop in a way.

I: Yeah.

I: Okay.

P: Yeah, so I would be a bit concerned about that.

P: But generally, like when I check the content, most of the time, it's correct.

P: Yeah.

P: Like professionally, privately, like what I've done with trips and stuff.

P: No.

I: Yeah, it hasn't been correct?

P: No, like for example, so I booked a [TICKET], and I was asking about the lineup and stuff.

P: And it was always wrong.

I: Oh yeah, makes sense.

P: Yeah, and that is quite easily accessible online if you look it up.

I: So have you thought about issues like bias in AI?

P: Like the AI being biased or me being biased?

I: Both.

P: Both?

P: Yeah, well, I'm a human.

P: I'm biased.

P: With the AI, I've tried to test it a couple of times.

P: Because, you know, like ChatGPT comes from the US.

P: So I was asking questions about opinions about the government and stuff.

P: And I found the AI, like ChatGPT in this case, not to be biased, but a bit behind, like a bit outdated in some outputs.

P: But otherwise, I feel like not that biased.

P: Perhaps a tiny bit.

P: generally quite unbiased

I: so how did you check for this bias

P: like for example i would ask questions about donald trump whether he's a felon whether he's convicted stuff like that whether he's sexist and stuff like that

P: So that's what I ask the AI.

P: Yeah.

I: Okay.

I: I think we can move on to the next question.

I: So this is client perspective and therapeutic alliance.

I: So how do you think clients respond to AI being used in your treatment?

P: Depends on the generation.

P: I think older people, younger people, I think they would be quite open to that.

I: Yeah, so I don't have anything for this —*thumbs up*— because we're not recording video, but you just said that younger people, they will probably be open to it, right?

I: And older people, not that much.

P: Yeah, I think they will be more distrusting because it's unfamiliar and it's new.

P: And historically, with all the experience that a lot of older people have,

P: understandably, they can be more distrusting.

P: Not everyone, but some people.

I: Yeah.

I: For this, we also want a score.

I: So if you could give a score of how willing people would be to use AI in their clinical or in their treatment.

P: Yeah, it depends on what population you look at.

P: But I think generally, considering that

P: we're moving forward,

P: I would say a six or seven.

I: Let's see.

I: So how might this affect the therapeutic alliance?

P: What do you mean by that specifically?

I: well i don't know if it's the same for you but i learned about the therapeutic alliance with you and your client you have some agreement right like you try and get better and stuff but if you're introducing ai to this relationship then this might change the dynamic.

P: hmm i think there is a possibility that this could introduce another instance but i also think that depends on the person because you know some ai is like for example chat gpt is made to sound like a human and that could create that even though it's technology of course but that could create that

P: sense that it's not just a two people alliance.

P: I think that could complicate things.

P: Definitely.

P: But that depends probably on which type of AI you use then.

I: Yeah.

P: Sorry, not which type, in what way.

I: Okay, so then let's see.

I: I think we can move to the last question.

I: So this is about professional identity, autonomy and future conditions.

I: Which aspects of your work should remain strictly human?

I: And what would you need to feel confident about using AI safely in practice?

P: like everything regarding organizing and communications with colleagues, as I said before, i think that can just be email communications, setting appointments and stuff.

P: I think that's totally useful.

P: I think the session itself should be human interactions.

P: yeah what else i think screening and stuff could be a combination of both and documenting i think as well because you need to check yeah i think it's not just one or another i think it's always a combination

I: So why do you think these aspects need to stay human or at least partially human?

P: Because I think some things you cannot replace by technology.

P: Like it's very unscientific the way I'll probably say that.

P: But just that feeling that you have and the expressions and stuff that you know that you have in human interactions.

P: It's like these very subtle things that technology cannot do yeah that sounds non-scientific

I: no i mean this is a perfectly fine answer okay so that's kind of it do you have anything else to add or anything

P: nothing.